



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فوزيه محمد السيد احمد
المحافظة : الجيزة
الحى / المدينة : العمرانية
رقم البطاقة : 26005161201243 رقم التليفون : 01022327388
العمر (تاريخ الميلاد) : 16-05-1960 المهنة : ربه منزل
العنوان : مصطفى بدوي
تاريخ تسجيل الإستمارة : 04-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- / د. / ----- / د. / ----- / د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فوزيه محمد السيد احمد National ID: 26005161201243	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

فوزية

محمد السيد احمد شبيكه

١٩ ش مصطفى بدوي، العمرانية الغربية
العمرانية - الجيزة

٢٦٠٠٥١٦١٢٠١٢٤٣

KQ8244417

٢٠٢٤/٠٧ ٢٦٠٠٥١٦١٢٠١٢٤٣

حاصلة على بكالوريوس زراعة

أنثى مسلمة متزوجة

محمد عطيه احمد الوكيل

البطاقة سارية حتى ٢٠٣١/٠٧/٢٠

الوراق على السيسى

Patient Name :	الأستاذة / فوزية محمد السيد أحمد	Request No. :	13006365
Patient No. :	26005161201243	Request Date :	06/05/2026
Path No. :	SR-547/26	Verification Date :	12/05/2026
Sex :	Female	Financial Category :	جوازات رومسي - نفقة الدولة
Age :	65 Y	Department :	Out Patient
Referring Doc :		Registered by :	.marwa mohamed youss

Pathology Report

Clinical Data: Left axillary lymph node in a patient with pathologically proven right breast cancer.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (332/26) and two referred slides including one (H&E) stained slide labelled by (1048459) and one immunostained slide for (SISH).

Microscopic Examination:

* Histopathological examination reveals cores of nodal tissue showing infiltration by tumor formed of malignant epithelial cells showing mild nuclear pleomorphism and eosinophilic cytoplasm.

** Review of the referred immunostained slide:

- SISH: Amplified.

(Average Her2 copy No.: (4.3), Her2/CEP17 ratio: (3.3)).

Diagnosis:

Left axillary lymph node, Referred block & slides (Tru-cut biopsy):

- Metastatic carcinoma, likely of breast origin.

IPT:

- SISH: Amplified, Group I.

(Average Her2 copy No.: (4.3), Her2/CEP17 ratio: (3.3)).

ISH groups:

The ratio is calculated and the results is interpreted as:

• Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.

Verified by :

Dr.Duaa Mohamed



Name: فوزية محمد السيد احمد		GUSTAVE ROUSSY	
MR. : 3633			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها			
The patient is a 66-year-old individual who is post-menopausal. There is no reported family history of breast or related malignancies and no documented co-morbidities. The core needle biopsy demonstrated invasive ductal carcinoma, grade II, bilateral, lateral. Immunohistochemistry showed estrogen-receptor positivity 8/8, progesterone-receptor positivity 6/8, HER2 score +2 with amplification on SISH, and Ki-67 of 40 %. Clinically, the patient presented in March 2022 with a right-breast mass classified as cT2N3 with infraclavicular lymph node involvement, consistent with moderately differentiated IDC (GII-II). Initial treatment comprised neoadjuvant chemotherapy (EC for approximately 12 weeks followed by Taxol), after which the patient underwent right-sided modified radical mastectomy and postoperative radiotherapy completed in December 2022. Subsequent adjuvant therapy included an aromatase inhibitor (Arimidex) and zoledronic acid (Zometa) every six months. In February 2026 the patient developed left-breast heaviness and erythema with left axillary lymph node involvement; biopsy confirmed metastatic mammary ductal carcinoma (GATA3 + , ER 8/8, PR 6/8, HER2 +2 equivocal but amplified, Ki-67 40 %). Work-up revealed bone metastases, and a PET-CT performed on 22-08-2026 demonstrated regression after initiation of Taxol and Herceptin. The referring team recommended continuation of Herceptin intravenous infusion together with an aromatase inhibitor (Femara) and zoledronic acid. The multidisciplinary team concluded that the disease represents metastatic HER2-positive breast cancer involving lymph nodes and bone, and advised resuming Taxol/Herceptin if tolerated without significant peripheral neuropathy; otherwise, the patient should be maintained on Herceptin with tamoxifen (given prior aromatase-inhibitor resistance) with close monitoring of therapeutic response.			
تحريراً في ,			
		التاريخ : 23-09-2026	الوقت :
		الطبيب المعالج :	
يعتمد ,			



PATHOLOGY & CYTOLOGY REPORT		
Patient Name: فوزية محمد السيد	Sex: F	Age: 66 y
Referred by: Prof. Dr. Lobna Salah	Dept.: Oncology	
Code: 1242310	Report No.: 26-366	

Immunohistochemistry Report

Specimen(s): Referred paraffin block (labeled as 332-26)

Clinical Diagnosis: Left breast carcinoma

Pathology report no 332-26: Core biopsy, left axillary lymph node:

Positive for metastatic carcinoma

Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by monoclonal antibodies and detection kit using Ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections were treated against GATA3, ER, PR and HER-2 neu & Ki67

Results:

GATA3: Positive staining of tumor cells, confirming metastatic mammary ductal carcinoma

- Estrogen Receptors: Positive staining of 90% of tumor cells, strong intensity. Allred score 8/8
- Progesterone Receptors: Positive staining of 30% of tumor cells, strong intensity. Allred score 6/8
- HER-2 neu: Equivocal (score 2+), for ISH.
- Ki-67 proliferation index 40% of tumor cells

Dr. Nedal Hegazy, MD.
Prof. of Pathology, Ain ShamsU.

Dr. Magda S. El Monayeri, MD.
Prof. of Pathology, Ain ShamsU.

Date: 9/04/2026

١٣٦٣٣

Patient Name :	الأمستاده / فوزيه محمد السيد احمد	Request No. :	13006365
Patient No. :	26005161201243	Request Date :	06/05/2026
Path No. :	SR-547/26	Verification Date :	12/05/2026
Sex :	Female	Financial Category :	جوستاف رومى - نفقة الدولة
Age :	65 Y	Department :	Out Patient
Referring Doc :		Registered by :	marwa mohamed youss

Pathology Report

- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Dr. Walaa Ibrahim Ghoneim.

Verified by :
Dr. Duaa Mohamed

Printing Date : 19/05/2026 10:55:40

Page 2 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / فوزية محمد السيد احمد	Request No. : 13004647
Patient No. : 26005161201243	Request Date : 07/12/2022
Path No. : SR- 951/22	Verification Date : 27/12/2022
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 62 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical data: Right breast cancer, PNAC
Nature of specimen: Slide Revision, (Axillary node).

Gross Examination:

- One block marked by (1372/22).
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Metastatic deposits of breast ductal origin.
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
- PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
- Her2: No membranous reaction in tumor cells (score 0)
- Ki67: 15% nuclear reaction of tumor cells

Diagnosis:

Right axillary LNs, Slide revision:
Positive lymph nodes metastases of breast ductal origin
IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 15% of tumor cells.

Prof. Dr. Manal Elmahdy
Dr. Hebatallah Mostafa

Verified by :
Dr.Marwa mahmoud

Printing Date : 28/12/2022 12:03:08

Page 1 of 2

Printing By : marwa mohamed youss

Patient Name	: الأستاذ / فوزيه محمد السيد احمد	Request No.	: 13005665
Patient No.	: 26005161201243	Request Date	: 01/04/2026
Path No.	: 847/26	Verification Date	: 15/04/2026
Sex	: Female	Financial Category	: جوماتاف روسي - نفقة الدولة
Age	: 65 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right axillary tail suspicious mass.

Nature of Specimen: Tru-cut biopsy, collected on 31/3/2026, at 13:45 , Cold ischemia time: 20 seconds.

Gross Examination:

- Four grey white tissue cores, the largest core measured 1.5 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma with cribriform differentiation.
- * Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.
- * Necrosis: Focal.

Diagnosis:

Right axillary tail suspicious mass, Tru-cut biopsy:

- Invasive duct carcinoma with cribriform features, Grade 2.
- For further IHC (ER, PR, Her2, Ki67& P63).

Signature

Prof Dr. Maha Guimeil.
Dr. Heba El Shafeey.

Verified by :

Dr Maha Guimie

Printing Date : 16/04/2026 12:54:56

Page 1 of 1

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / فوزيه محمد السيد أحمد	Request No.	: 13005665
Patient No.	: 26005161201243	Request Date	: 01/04/2026
Path No.	: 847/26	Verification Date	: 16/04/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 65 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right axillary tail suspicious mass diagnosed invasive duct carcinoma with cribriform differentiation, Grade 2.

Nature of Specimen: Tru-cut biopsy, collected on 31/3/2026, at 13:45 pm, Cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; P63 clone (4A4)□, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Results:

* IPT (All controls showed appropriate reactivity):

- P63: Negative.

- ER: Strong nuclear reaction in 85% of tumor cells (score 8/8).

- PR: Moderate nuclear reaction in 95% of tumor cells (score 8/8).

- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).

- SISH: Not amplified.

(Average Her2 copy No.: (2.8), Her2/CEP17 ratio: (1.86)).

- Ki67%: 60%.

Diagnosis:

Right axillary tail mass, Tru-cut biopsy:

IPT:

- ER: Positive (score 8/8).

- PR: Positive (score 8/8).

- Her2: Equivocal (score 2+).

SISH: Not amplified, Group V.

(Average Her2 copy No.: (2.8), Her2/CEP17 ratio: (1.86)).

Verified by :

Dr.Duaa Mohamed

Printing Date : 16/04/2026 12:55:00

Page 1 of 3

Printed By : .marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mias El Qadema - Cairo, Egypt

Patient Name الأستاذ / فوزيه محمد السيد احمد
Patient No. 26005161201243
Path No. 847/26
Sex Female
Age 65 Y
Referring Doc.

Request No. 13005665
Request Date 01/04/2026
Verification Date 16/04/2026
Financial Category جوساث روسي - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

- Ki67%: 60%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).

Verified by :

Dr.Duaa Mohamed

Printing Date : 16/04/2026 12:55:00

Page 2 of 3

Printed By : .marwa mohamed yousse



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name الأستاذ / فوزيه محمد السيد احمد
Patient No. 26005161201243
Path No. 847/26
Sex Female
Age 65 Y
Referring Doc.

Request No. 13005665
Request Date 01/04/2026
Verification Date 16/04/2026
Financial Category جوستاف روسي - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

• Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ebrahim Ghoneim.

Dr. Heba El Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 16/04/2026 12:55:00

Page 3 of 3

Printed By : .marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Visit Number:	61126507116	Client Name:	
Patient Name:	Fawzya Mohamed El-Sayed	Referred By Prof:	
Age / Sex:	65 year / Female	Exam Date:	06 Apr 2026 00:00
Patient ID:	2140659883 Acc:1870759	Printed:	07/04/2026 1:34:44 PM

PT

Whole Body PET/CT Examination

History and Clinical Data:-

Patient data:-

- Female patient, 65 years old, Weight: 83 Kg, Height: 160 cm, Blood Glucose level: 119 mg/dl.

Radiopharmaceutical:-

- F-18-FDG 5 mCi injected I.V.

Clinical background:-

- Reported case of right breast cancer, underwent mastectomy at 2022, received RTH.
- Recently complaining of left breast skin edema and axillary lymphadenopathy.
- No available studies for comparison.

Purpose of the study: -

- Assessment and metastatic workup.

TECHNIQUE: -

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~ 2.4
- Mediastinal pool average SUV ~ 1.9

REPORT:-

Review of the PET, CT and the fused images revealed:

1) Loco regional (breasts and axillae):

a) Right side; status post-operative (right mastectomy) showing:

- The operative bed showing minimal soft tissue thick sheets (**post-operative sequel**), with no metabolically active mass lesion.
- Few metabolically active right axillary lymph nodes are noted measuring up to 2.6 cm with max. SUV~ 8.36 with mild stranding of surrounding fat planes.

b) Left side:

- Low grade metabolically active diffuse parenchyma condensation and edema with max. SUV~ 2.47. No definite FDG avid solid mass.

Visit Number:	61126507116	Client Name:	
Patient Name:	Fawzya Mohamed El-Sayed	Referred By Prof:	
Age / Sex:	65 year / Female	Exam Date:	06 Apr 2026 00:00
Patient ID:	2140659883 Acc:1870759	Printed:	07/04/2026 1:34:44 PM

PT

- Low grade metabolically active left breast skin thickening reaching up to 1.5 cm in thickness with max. SUV~ 3.24.
- Multiple prominent left axillary and retro-pectoral lymph nodes noted, some of them showing FDG uptake measuring up to 1.7 cm with max. SUV~ 6.07 (at the axillary lymph node).

II) Distant nodal:

- Few metabolically active left infra-clavicular and internal mammary lymph nodes are noted measuring up to 2 cm with max. SUV~ 5.63 at lymph node (which seen indenting/inseparable from left upper chest wall muscle).
- Few low grade metabolically active bilateral lower deep cervical and bilateral supraclavicular lymph nodes measuring up to 1.1 cm with max. SUV~ 2.66 (at left group Vb lymph node).
- Multiple metabolically active superior mediastinal, pre-vascular, pre-tracheal, sub-carinal, bilateral hilar, bilateral internal mammary, posterior mediastinal, right cardio-phrenic and right epiphrenic lymph nodes, measuring up to 2.5 cm with max. SUV~ 4.7 (at the right hilar lymph node).
- Metabolically active porto-caval, celiac and porta-hepatis, aorto-caval and bilateral (common, internal and external) iliac as well as inguinal lymph nodes measuring up to 3 cm with max. SUV~ 5.86 at the right external iliac node), while the highest max. SUV~ 9.33 is recorded at 2.5 cm right internal iliac lymph node.

III) Osseous:

- Multiple metabolically active marrow-based and faintly sclerotic osseous lesions involving multilevel dorso-lumbar, S1, vertebrae, multilevel left ribs, sternum, both iliac bones, acetabular roof, both femoral neck and proximal shafts seen with max. SUV~ 5.84 (at L1 lesion).
- Spondylodegenerative changes.
- Right iliac bone island noted.

IV) The rest of the body:

- Moderately enlarged liver with no metabolically active (FDG avid) hepatic focal lesions.
- Average sized spleen with no metabolically active (FDG avid) splenic focal lesions.
- No metabolically active pulmonary nodules or masses seen. Mild low-grade FDG avid right pleural effusion.

Visit Number:	61126507116	Client Name:	
Patient Name:	Fawzya Mohamed El-Sayed	Referred By Prof:	
Age / Sex:	65 year / Female	Exam Date:	06 Apr 2026 00:00
Patient ID:	2140659883 Acc:1870759	Printed:	07/04/2026 1:34:44 PM

PT

Side Findings:

- Left sided porta-cath noted.
- Right maxillary sinusitis with possible fungal colonization.
- Gravitational haze.
- Bilateral lower and middle lobar atelectatic bands.
- Calcular gallbladder.
- Small left renal cortical cyst.
- Low grade metabolically active bilateral gluteal subcutaneous nodules and fat stranding; likely related to IM injection site.
- IUD is seen within the uterus

V) The rest of PET / CT study appears normal.

VI) Normal physiological tracer biodistribution is seen over cerebral hemispheres, heart, bowel and urinary tract.

CONCLUSIO:

Reported case of right breast cancer (following surgery and therapy) showing:

- No metabolically active (FDG avid) local tumoral recurrence within the right breast operative bed.
- Low grade metabolically active left breast parenchymal condensation/edema and skin thickening, for clinical correlation.
- Multiple metabolically active (FDG avid) regional right axillary and multiple distant nodal metastatic deposits; as described.
- Multiple metabolically active osseous metastatic deposits.
- Low-grade metabolically active right pleural effusion.

Prof. Ahmed Wafaie MD, FRCR
Dr. Samar Haddad MSc FEBR

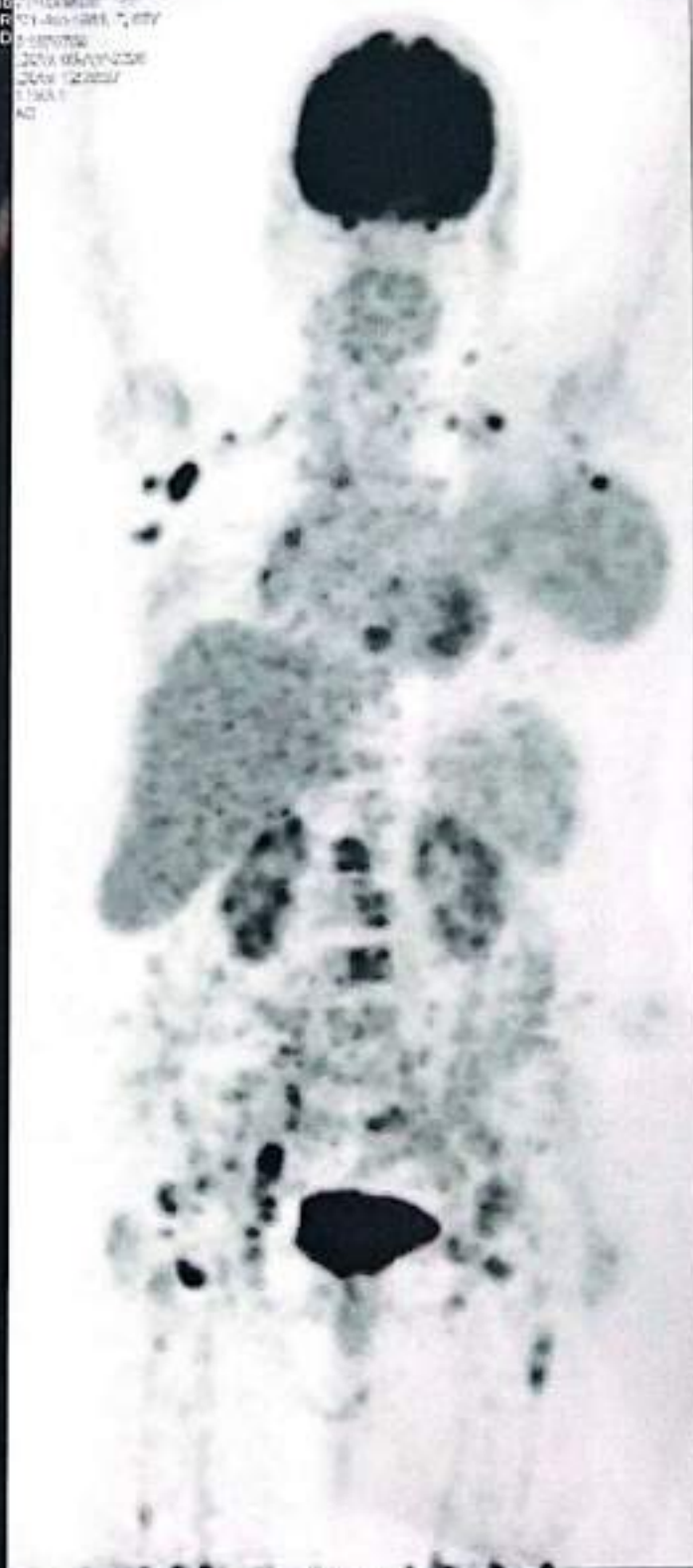
Fawzya, Mohamed, El-Sayed
 2140659885
 01-Jan-18, F. 65Y
 #1870759
 06-Apr-2020
 12:45:59 AM
 608 MB, 1
 Mmap, FUSED VRT MIP

IDH Mohamedan Fawzya, Mohamed, ElSayed
 Ref: Khalid Abd Ragab
 Biograph16_Hotzone 3D
 VJ300

#1870759
 2019 04/07/2020
 2019 12/20/2017
 11963.1
 AQ



SENSITIVE



SENSITIVE

Fawzya, Mohamed, El-Sayed

2140659883

*01-Jan-1961, F, 65Y

1870759

06-Apr-2026

12:46:12.00

1072 IMA 29

AC

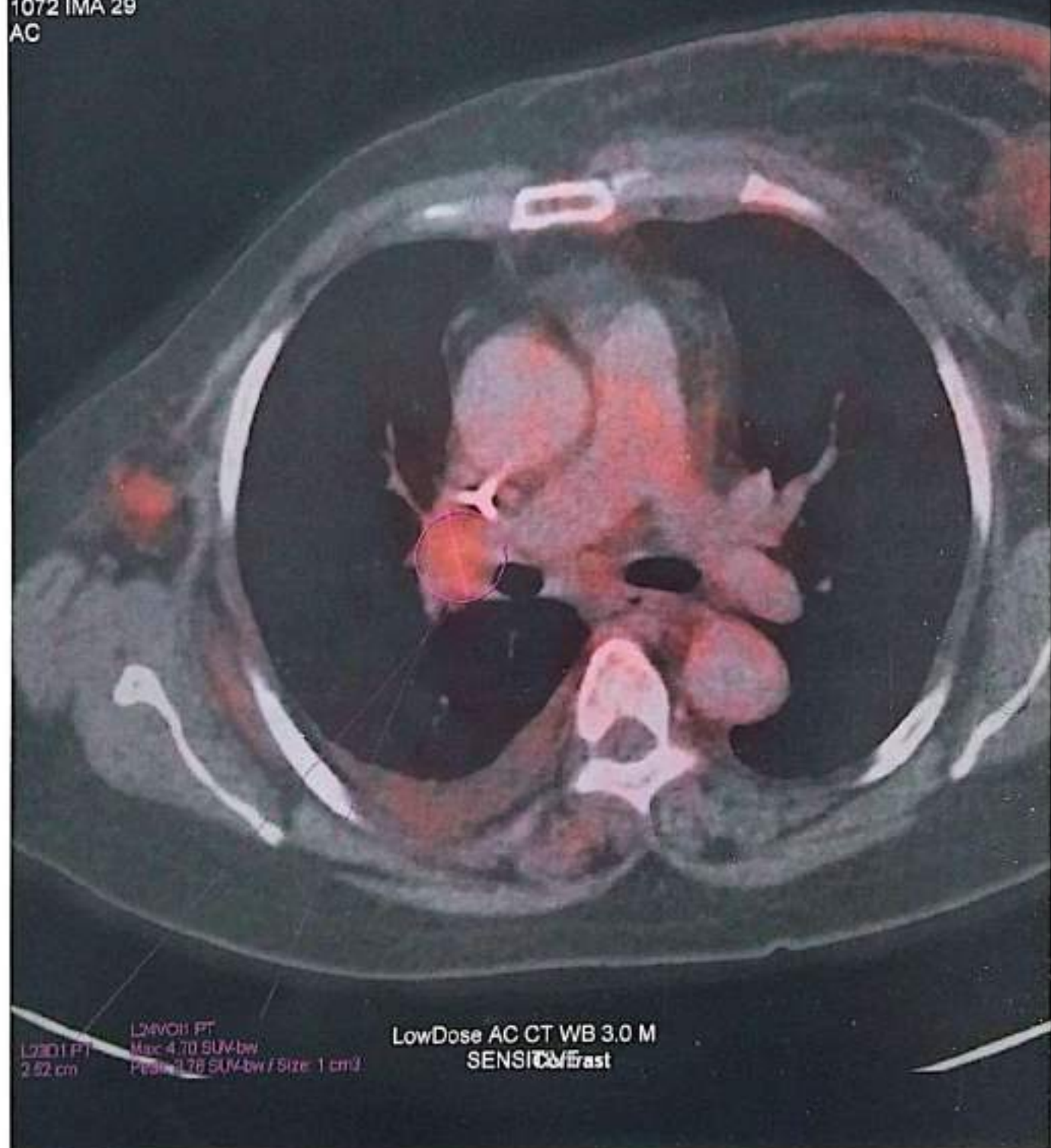
A

IDH Mohandesen

Ref.: Khaled Abd Ragab

syngo.via

syngo.PET&CT Oncology 8.17.25...



L23D1 (F)
2.52 cm

L23VOH (F)
Max: 4.70 SUV-bw
PSMA: 3.78 SUV-bw / Size: 1 cm3

LowDose AC CT WB 3.0 M
SENICOREast

Fawzya, Mohamed, El-Sayed
2140659883
*01-Jan-1961, F, 65Y
1870759
06-Apr-2026
12:46:12.00
1072 IMA 35
AC

A

IDH Mohandesen
Ref.: Khaled Abd Ragab
syngo.via
syngo.PET&CT Oncology 8.17.25...



Fawzya, Mohamed, El-Sayed

2140659883

*01-Jan-1961, F, 65Y

1870759

06-Apr-2026

12:46:12.00

1072 IMA 36

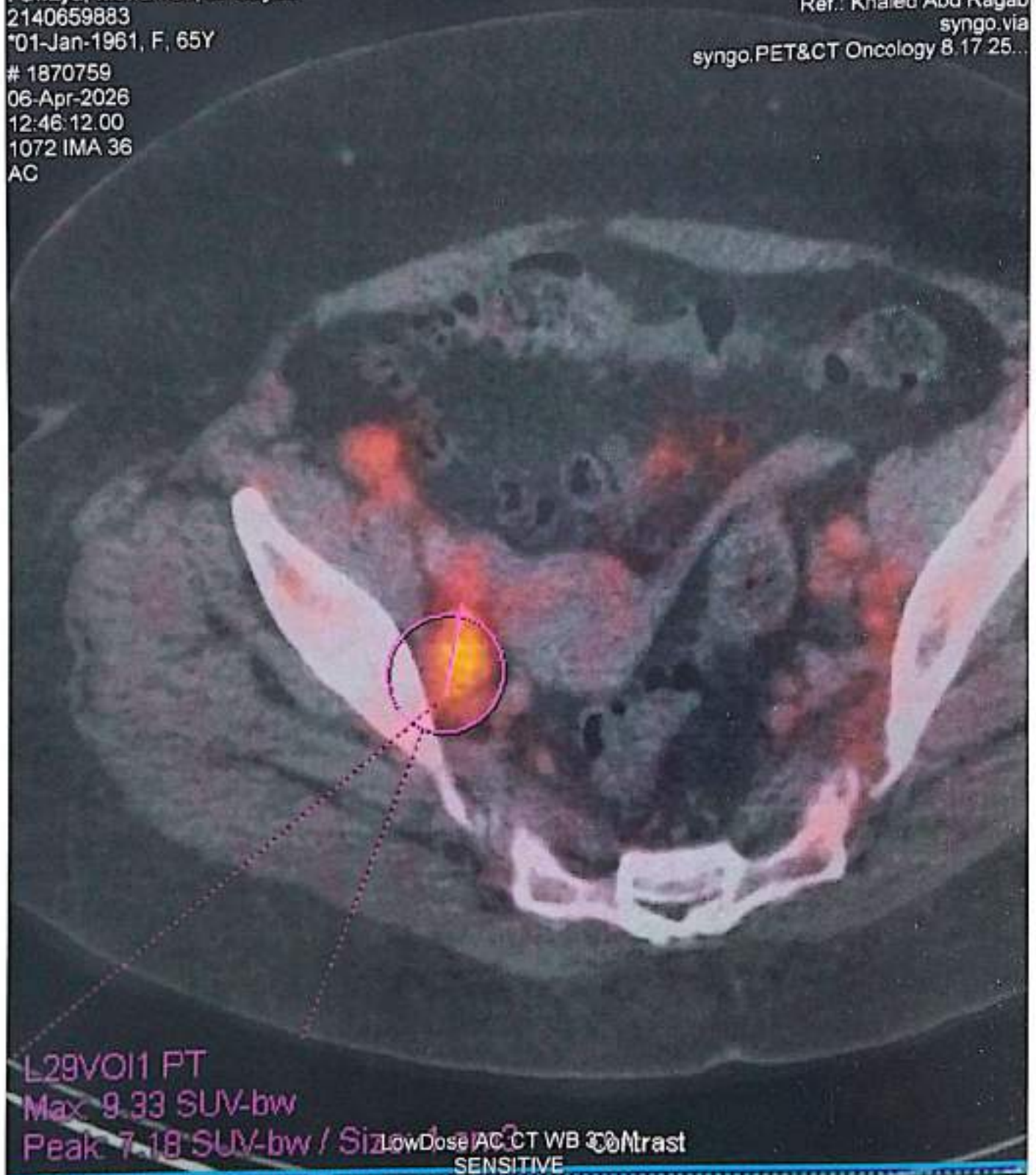
AC

IDH Mohandesen

Ref.: Khaled Abd Ragab

syngo.via

syngo.PET&CT Oncology 8.17.25...



Fawzya, Mohamed, El-Sayed
2140659883
*01-Jan-1961, F, 65Y
1870759
06-Apr-2026
12:59:25.41
1072 IMA 46
MPR

H

IDH Mohandesen
Ref.: Khaled Abd Ragab
syngo.via
syngo.PET&CT Oncology 8.17.25..

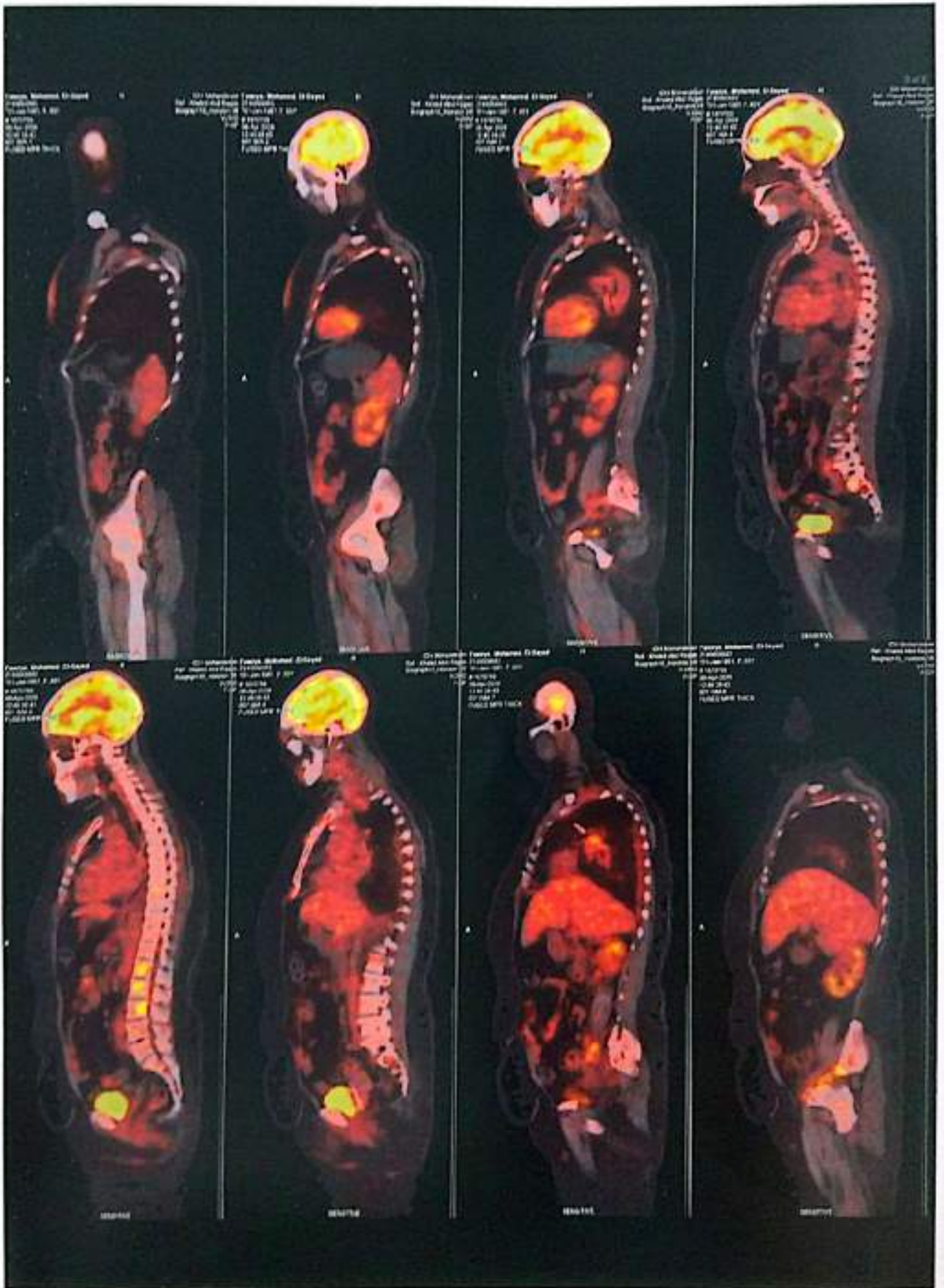
A

L45V011 PT
Max: 5.84 SUV/bw
Peak: 4.74 SUV/bw / Size:

10cm

Contrast
SENSITIVE





EGYPT AIR HOSPITAL

REFERRED BY PROF:

DATE: 18/5/2026

PATIENT NAME: Fawzeya Mohammed El Sayed

AGE : 66Y

CODE: 01242310

Echo - Doppler Report

	Patient	Normal		Patient	Normal
<u>Left ventricle</u>			<u>Aortic root & valve</u>		
LVEDD	53	38 -56 mm	Diameter		20-39 mm
LVESD	33	22 -40 mm	Cusp opening	29	15-26 mm
F.S.	37%	25 - 45 %			
E.F.	67%	50 - 77 %	<u>Left Atrium</u>		
<u>Septum</u>	10	7 -11mm	Diameter	41	15 -40 mm
<u>Posterior wall</u>	11	7 -11mm	LA / AO		0.8 -1.3

Mitral Valve:

MORPHOLOGY:

Mobility:0

Thickness:0

Calcification: 0

Subvalvular apparatus:0

E/A RATIO: reversed

Aortic valve: Sclerotic

Pulmonary valve: NORMAL

Tricuspid valve: NORMAL

Pericardium: NORMAL

Conclusion

- Normal LV cavity with good LV systolic function.
- Dilated LA dimension.
- Diastolic dysfunction (reversed E/A ratio), trivial MR.
- Sclerotic aorta with no significant gradient.
- No segmental walls motion abnormalities could be detected at rest.
- Trivial TR with no evidence of pulmonary hypertension.
- Normal Rt side of the heart.
- Intact interatrial & interventricular septae.
- No Intra-cardiac masses or thrombi could be detected by TTE.
- Normal pericardium.

Dr /Hend El Deeb M.D

Ass.prof. of cardiology

اسم الفرع خاتم المرسلين
مدير الفرع د. ريهام محروس النيفي
مدير الوحدة ا.د. منال كمال
طبيب الوحدة د. سمها حاتم

Visit Number: 76926508029 Registered: 05-09-2026 09:13:37
Patient Name: فريدة محمد السيد احمد Collected: 05-09-2026 09:13:58
Age / Sex: 66 Year / Female Authenticated: 05-09-2026 15:53:46
Referred By: محمد الكيلاني Prof: Printed: 05-09-2026 20:14:56
Client Name: 0

TEST NAME	RESULT	UNIT	REFERENCE RANGE
Liver Function Tests			
SGPT (ALT)	16	U/L	0 - 33
SGOT (AST)	15	U/L	0 - 32

Kidney Function Tests			
Serum Creatinine	0.59	mg/dL	0.5 - 1.1
Serum Urea	28	mg/dL	17 - 49
Serum Calcium	9.6	mg/dL	8.6 - 10.2

Dr. Manal Kamal
Professor of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University



خضرم 30% على جميع أنواع الأشعة للقرء محدودة لا يسري مع التعقيدات ولا أي خصوصيات أخرى

خادم البرنامج
اسم المريض
مختبر القصر
مختبر الوحدة
طبيب الوحدة
د. خالد عبد الله
د. رانيا سليمان
د. بديع فاروق

Visit Number: 76926508029
Patient Name: فوزية محمد السيد احمد
Age / Sex: 66 Year / Female
Referred By: محمد الكلاسي
Client Name: 0

Registered 05-09-2026 09:13:37
Collected 05-09-2026 09:13:58
Authenticated 05-09-2026 15:37:08
Reported 05-09-2026 20:14:58

HEMATOLOGY REPORT

TEST NAME	RESULT	UNIT	REFERENCE RANGE
-----------	--------	------	-----------------

Complete Blood Picture

Haemoglobin (EDTA Blood)	12.2	g/dL	11.5 - 15.5
Haematocrit (PCV)	39.4	%	36 - 45
RBCs Count (EDTA Blood)	4.98	Millions / cmm	4 - 5.2
MCV	L 79.1	fL	80 - 100
MCH	L 24.5	pg	27 - 33
MCHC	31.0	g/dL	31 - 37
RDW-CV	H 18.5	%	11.5 - 15
Platelet Count	293	Thousands / cmm	150 - 450
Total Leucocytic Count (EDTA Blood)	L 3.9	Thousands / cmm	4 - 11
Percent Values			
Neutrophils	64.0	%	2.51
Lymphocytes	27.3	%	1.07
Monocytes	7.4	%	0.29
Eosinophils	0.0	%	0.00
Basophils	1.3	%	0.05
Other Cells			
Absolute Values			
Neutrophils	2.51	x10 ⁹ /L	2 - 7
Lymphocytes	1.07	x10 ⁹ /L	1 - 4.8
Monocytes	0.29	x10 ⁹ /L	0.2 - 1
Eosinophils	0.00	x10 ⁹ /L	0.1 - 0.45
Basophils	0.05	x10 ⁹ /L	0 - 0.1

Comment:
RBCs show mild hypochromia and anisocytosis. Mild leucopenia.
Follow up is recommended.

Dr. Lamyaa Soliman
Assistant professor of clinical pathology
Faculty of medicine, Cairo university

Lamyaa Soliman

اسم الفرع خاتم المرسلين
مدير الفرع د. ريهام محروس النيفي
مدير الوحدة ا.د. منال كمال
طبيب الوحدة د. سمها حاتم

Visit Number: 76926508029 Registered: 05-09-2026 09:13:37
Patient Name: فريدة محمد السيد احمد Collected: 05-09-2026 09:13:58
Age / Sex: 66 Year / Female Authenticated: 05-09-2026 15:53:46
Referred By: محمد الكيلاني Prof: Printed: 05-09-2026 20:14:56
Client Name: 0

TEST NAME	RESULT	UNIT	REFERENCE RANGE
Liver Function Tests			
SGPT (ALT)	16	U/L	0 - 33
SGOT (AST)	15	U/L	0 - 32

Kidney Function Tests			
Serum Creatinine	0.59	mg/dL	0.5 - 1.1
Serum Urea	28	mg/dL	17 - 49
Serum Calcium	9.6	mg/dL	8.6 - 10.2

Dr. Manal Kamal
Professor of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University



خضرم 30% على جميع أنواع الأشعة للقرء محدودة لا يسري مع التعقيدات ولا أي خصوصيات أخرى

خادم البرنامج
اسم المريض
مختبر القصر
مختبر الوحدة
طبيب الوحدة
د. خالد عبد الله
د. رانيا سليمان
د. نيفين فاروق

Visit Number: 76926508029
Patient Name: فوزية محمد السيد احمد
Age / Sex: 66 Year / Female
Referred By: محبة الكلاشي Prof.
Client Name: 0

Registered 05-09-2026 09:13:37
Collected 05-09-2026 09:13:58
Authenticated 05-09-2026 15:37:08
Reported 05-09-2026 20:14:58

HEMATOLOGY REPORT

TEST NAME	RESULT	UNIT	REFERENCE RANGE
-----------	--------	------	-----------------

Complete Blood Picture

Haemoglobin (EDTA Blood)	12.2	g/dL	11.5 - 15.5
Haematocrit (PCV)	39.4	%	36 - 45
RBCs Count (EDTA Blood)	4.98	Millions / cmm	4 - 5.2
MCV	L 79.1	fL	80 - 100
MCH	L 24.5	pg	27 - 33
MCHC	31.0	g/dL	31 - 37
RDW-CV	H 18.5	%	11.5 - 15
Platelet Count	293	Thousands / cmm	150 - 450
Total Leucocytic Count (EDTA Blood)	L 3.9	Thousands / cmm	4 - 11
Percent Values			
Neutrophils	64.0	%	2.51
Lymphocytes	27.3	%	1.07
Monocytes	7.4	%	0.29
Eosinophils	0.0	%	0.00
Basophils	1.3	%	0.05
Other Cells			
Absolute Values			
Neutrophils	2.51	x10 ⁹ /L	2 - 7
Lymphocytes	1.07	x10 ⁹ /L	1 - 4.8
Monocytes	0.29	x10 ⁹ /L	0.2 - 1
Eosinophils	0.00	x10 ⁹ /L	0.1 - 0.45
Basophils	0.05	x10 ⁹ /L	0 - 0.1

Comment:
RBCs show mild hypochromia and anisocytosis. Mild leucopenia.
Follow up is recommended.

Dr. Lamyaa Soliman
Assistant professor of clinical pathology
Faculty of medicine, Cairo university

Lamyaa Soliman